

WHITMORE, Harold James

DOB: March 8, 1945 | Age: 76 yrs | Sex: Male | MRN: SMC-04471932
Encounter Date: May 3, 2021 | Provider: Laura Simmons, MD — Emergency Medicine | Doc ID: SMC-2021-0503-ED

Emergency Department Visit

Transient Right Arm Weakness & Slurred Speech — TIA

HPI

76-year-old man with sudden onset right arm weakness and slurred speech lasting ~25 minutes, fully resolved before arrival. No headache, visual loss, or loss of consciousness. On apixaban for AF (reports good adherence). Last known well 90 minutes prior.

Vital Signs

BP (mmHg)	HR (bpm)	RR	Temp	SpO2	Weight	BMI
162/94	78 irreg	16	36.7 C	97% RA	92.0 kg	30.0

Neurologic Examination (on arrival)

Alert, oriented. Speech now fluent without dysarthria. Cranial nerves intact. Motor 5/5 all limbs, symmetric. No drift, sensory loss, or ataxia. NIHSS 0. ABCD2 score 5 (high risk).

ED Labs

Test	Result	Units	Reference Range	Flag
Glucose	148	mg/dL	70 - 140	H
Creatinine	1.8	mg/dL	0.7 - 1.3	H
eGFR (CKD-EPI)	38	mL/min/1.73 m2	> 60	L
INR	1.1		N/A (on DOAC)	
LDL cholesterol	70	mg/dL	< 55 (very high risk)	H

Imaging

CT head non-contrast: no acute hemorrhage or established infarct. CT angiography head/neck: 50-69% stenosis of the proximal left internal carotid artery; patent intracranial vessels. MRI brain (next morning): tiny acute cortical diffusion-restricting focus in left frontal region — consistent with small embolic event.

Assessment & Disposition

High-risk TIA / minor ischemic event, likely cardioembolic (AF) and/or artery-to-artery from left ICA stenosis. Already anticoagulated. Admitted to stroke unit for observation, neurology consult, and secondary prevention optimization.

Plan

- Continue apixaban (confirm adherence; anticoagulation already optimal for AF — do NOT add antiplatelet given bleeding risk unless symptomatic carotid revascularization).
- Intensify lipid lowering: add ezetimibe 10 mg to atorvastatin 80 mg (target LDL <55).
- Optimize BP (goal <130/80).
- Carotid duplex to confirm stenosis; vascular surgery opinion (left ICA 50-69%, currently medical management).
- Neurology follow-up; swallow and PT/OT assessment.